Bioresearch Monitoring (BIMO) Data Reviewer's Guide (BDRG)

Study Name: TROPIC Re-Analysis (Study EFC6193 / XRP6258, NCT00417079)

Compound: Mitoxantrone (MP) control arm -- real, de-identified cohort (N=371)

Standard: FDA Bioresearch Monitoring Technical Conformance Guide (clinsite, Appendix 3)

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1. Purpose

The summary-level clinical-site dataset (clinsite) supports the FDA Office of Scientific Investigations (OSI) / BIMO program. It aggregates subject-level enrollment, disposition, and safety experience to the study-site level so that reviewers can prioritise clinical sites for on-site inspection. It is not an analysis dataset and is not part of the ADaM define.xml; per the BIMO Technical Conformance Guide it is delivered with its own data-definition documentation (this BDRG) under m5/datasets/tropic/bimo/.

This guide follows the structure recommended by the PHUSE BIMO Data Reviewer's Guide Completion Guidelines.

2. Honest Scope vs. the Full BIMO TCG `clinsite` Specification

*The full BIMO TCG Appendix-3 `clinsite` structure specifies **~39 site-level variables** (investigator identity/address/contact, country, screen/randomized/treated/completed/discontinued counts, protocol-deviation counts, primary-endpoint contribution, financial-disclosure flags, etc.). This portfolio implements the **subset that is HONESTLY DERIVABLE** from the public, de-identified TROPIC release (Project Data Sphere). It deliberately does **not** fabricate variables the source cannot support. This is an *illustrative BIMO subset*, not a submission-complete clinsite.*

Variables intentionally NOT populated, and why:

Omitted BIMO TCG content	Reason it is not derivable from this source
Investigator name / address / phone / email	The de-identified PDS release carries **no** principal-investigator identity. `INVNAM` below is a clearly-labelled **synthetic placeholder** (`PI_<siteid>`), never a real investigator.
`COUNTRY` / site geography	Not present in the de-identified release.
Important / significant protocol deviations	No SDTM `DV` (protocol deviations) domain is available in the public release.

Omitted BIMO TCG content	Reason it is not derivable from this source
Screened / completed / discontinued counts	DS disposition reasons are not separable into screen-fail vs. completion vs. discontinuation in the de-identified release.
Financial disclosure	Not applicable to a public secondary-use dataset.

A production BIMO package would populate the full Appendix-3 structure from the sponsor's operational/CTMS data; the derivation pattern demonstrated here (subject-level ? site-level roll-up, joined across populations and safety) is the transferable skill.

3. Variables Delivered (`clinsite`, one row per study site; 69 sites)

Variable	Label	Derivation		
`STUDYID`	Study Identifier	ADSL `STUDYID`		
`SITEID`	Study Site Identifier	ADSL `SITEID` (group key)		
`INVNAM`	Principal Investigator (**SYNTHETIC** placeholder)	`"PI_"		SITEID` -- see §2
`N_RAND`	Number of Subjects Randomized	`COUNT(DISTINCT USUBJID)` per site		
`N_SAF`	Number of Subjects Treated (Safety Population)	`SAFFL='Y'` per site		
`N_ITT`	Number of Subjects in ITT Population	`ITTFL='Y'` per site		
`N_PPROT`	Number of Subjects in Per-Protocol Population	`PPROTFL='Y'` per site		
`N_DEATH`	Number of Subjects Who Died	`DTHFL='Y'` per site		
`N_SAE`	Number of Subjects with a Serious AE	distinct `USUBJID` with ADAE `AESER='Y'`, routed to site via ADSL		

Variable	Label	Derivation
`N_TEAE`	Number of Subjects with a TEAE	distinct `USUBJID` with ADAE `TRTEMFL='Y'`, routed to site via ADSL

***Population note (ICH E9):** Randomized, Safety (treated), ITT, and Per-Protocol are **distinct analysis sets**. ITT is reported as `N_ITT` and is **never** relabelled "Efficacy Population" (a prior version mislabelled the ITT count as efficacy -- corrected). In this de-identified single-arm release all subjects are randomized, treated, ITT, and per-protocol, so those four counts coincide per site; the safety counts (`N_DEATH`, `N_SAE`, `N_TEAE`) vary by site and carry the inspection-prioritisation signal.*

4. Dual-Programming / Reconciliation

clinsite is double-programmed like the ADaM domains: produced by SAS (02_production_sas/B_bimo_generation.sas ? clinsite_prod.xpt) and independently by R (03_validation_r/v_bimo_validation.R ? clinsite_v.xpt), then reconciled cell-by-cell on the (STUDYID, SITEID) key by 05_reconciliation/cross_lang_audit.R. As with all domains, a genuine SAS?R reconciliation requires a run executed against a real SAS engine (sas_execution_mode = oda/local in 06_telemetry/pipeline_health.json); a sim-mode byte-copy reconciliation is tautological.

5. References

* FDA, *Bioresearch Monitoring Technical Conformance Guide* (clinsite, Appendix 3).
* PHUSE, *BIMO Data Reviewer's Guide (BDRG) Completion Guidelines*.
* PHUSE SA01, *Development of a standard BIMO process to create the clinsite dataset*.
* ICH E9, *Statistical Principles for Clinical Trials* (analysis-set definitions).